

Meeting Minutes: FDA Contact Center Optimization Project Industry Day

Date: March 5, 2026

Time: 9:30 AM - 12:00 PM

Location: FDA Headquarters – Great Room

FDA Leadership and Panel Members:

- Dr. Butler - Chief Operating Officer
- Shailesh Shah - Program Manager, Contact Center Optimization Project
- Melissa Shea - Deputy COO (Facilitator for Q&A)
- Sean Hanlon - Office of Digital Transformation (Security)
- Tonya Wilbon - CDRH Representative
- Josh Lehman - ODT Representative
- Mary Kremzner - CDER Representative
- Deborah Price - Foods & Cosmetic Information Center Representative
- Danni Gaines - ODT Representative
- Lilibeth Deato - Office of Acquisition and Grants Services (OAGS)
- Laura Alvey - Center for Veterinary Medicine (CVM)
- Melanie Keller - Incoming Chief Operating Officer (effective April 5)

I. Welcome and Opening Remarks

- **Introduction**
 - Meeting commenced at 9:30 AM
 - Purpose: Industry Day for Contact Center Optimization Project
 - Agenda overview provided
- **Opening Remarks (Dr. Butler)**
 - Key Announcements:
 - Dr. Butler announced retirement, effective with transition on April 5, 2026
 - Melanie Keller will assume Chief Operating Officer role
 - Melissa Shea will become Deputy COO
 - Mission Statement:
 - FDA operates 100+ intake pathways processing 200,000+ inquiries annually
 - Current systems are fragmented, limiting visibility and response time
 - Vision:
 - Single intelligent front door to the agency with enterprise visibility
 - Transformation Goals:
 - Unified contact center capability across 6 external-facing centers (Phase 1)
 - Enterprise CRM platform for signal detection and predictive insights
 - Tiered, intelligent engagement structure

- Self-service capabilities for stakeholders
- Consolidated intake with intelligent routing
- Business Value:
 - Earlier detection of emerging safety signals
 - Economies of scale through consolidation
 - Reduced technical debt
 - Dynamic resource allocation
 - Consistent stakeholder experience

II. Ground Rules

- Vendors must identify name and company before asking questions
- One follow-up question allowed if time permits
- Q&A exchanges limited to 2-3 minutes
- FDA will not discuss proprietary information, preferred vendors, source selection criteria, or budget-sensitive information
- Only written responses to formal questions are binding
- FDA reserves right to decline answering premature or inappropriate questions
- All Q&A will be shared with attendees post-event
- Session recorded and transcribed for documentation
- Q&A content may be incorporated into FAQ document
- Industry Day information is for guidance only and does not constitute solicitation or commitment

III. Project Overview and Goals

- **Vision and Architecture**
 - **Current State:**
 - Multiple intake pathways across FDA
 - Fragmented systems built for local needs
 - Limited enterprise-level pattern identification
 - **Future State:**
 - Single front door with unified operating model
 - Tiered structure (Tier 0-3)
 - Enterprise CRM platform
 - AI-powered chatbot capabilities
 - Integration with existing systems (Genesis telephony, Salesforce, Appian, ServiceNow)
- **Three Core Goals**
 - **Goal 1: Implement Tiered Contact Center Structure**
 - Tier 0 - Self-Service:
 - Update operational governance and best practices for FDA digital content
 - Conduct inventory of public FDA websites and consolidate redundant pages
 - Improve FDA digital content search functionality
 - Consolidate multiple intake forms and email addresses into unified intake tool (UIT)
 - Tier 1 - Initial Contact:

- Establish enterprise Tier 1 call center for current and anticipated volumes
 - Work with centers to define and script Tier 1 calls and escalation procedures
 - Require FDA-wide use of automated call distribution system
 - Route calls through single point of entry
- Tier 2 & 3 - Escalation and Expertise:
 - Maintain Tier 2 and Tier 3 calls in appropriate business units
 - Ensure specialists and experts maintain knowledge of specific products/policies
 - Tier 3 to remain with government subject matter experts
- **Goal 2: Form Business Advisory Group**
 - Establish small advisory group with representation from FDA centers, offices, and programs
 - Provide authoritative governance for long-term improvements
 - Advise on key project components including:
 - Workflow and system requirements
 - Vendor selection (RFI to RFQ)
 - Change management, communication, and training
 - Agency protocols and metrics
 - Maintain focus on public experience
 - Integrate Tier 0 changes into overall website plan
- **Goal 3: Establish CRM Governance**
 - Assess current state of FDA-wide CRM processes and workflow systems
 - Provide operational governance and best practices
 - Consolidate contracts, licenses, and new development
 - Enterprise CRM Solution - key requirements:
 - Comprehensive case and ticket management
 - Customer relationship and contact management
 - Tier structure and escalation management
 - Regulatory compliance and security
 - Reporting, analytics, and performance management
 - Integration and interoperability with single sign-on
 - User experience and interface
 - Scalability and future enhancement

IV. Question and Answer Session

Key Questions and Responses:

- Q: Tier 0 content updates and publishing platform integration
 - A: Vendors will work with existing team supporting content and publishing platform to make suggested implementation updates.
- Q: Lessons learned from prior modernization efforts

- A: Change management is critically important. Strong stakeholder communication and thorough discovery/assessment are essential.
- Q: Self-service scope and innovation boundaries
 - A: Think as broad and wide as possible. FDA does not want to limit innovation potential.
- Q: Center involvement in developing unified solution
 - A: All centers have representatives on the Integrated Product Team (IPT). Centers will evaluate responses and have voice in decision-making.
- Q: Assessment vs. solution development concerns
 - A: Assessment will determine if existing investments should be included or if new capabilities provide better value.
- Q: Tier 2 and Tier 3 staffing
 - A: Clarification provided: Tier 1 will be contractor-staffed. Tier 2 open to proposed solutions. Tier 3 will be government-staffed subject matter experts.
- Q: Solicitation timeline
 - A: Target release within next 60-90 days (fiscal year 2026). Leadership pushing for release this fiscal year.
- Q: Small business and subcontracting requirements
 - A: Still in development. FDA welcomes suggestions. Small business requirements will be included.
- Q: Change management evaluation
 - A: Yes, change management will be a significant evaluation factor in proposals.
- Q: Initial Operating Capability (IOC) expectations
 - A: Phase 1 (base year) should include operating capability, not just setup. Transition of multiple contact centers to be addressed in proposals.
- Q: AI integration and Elsa
 - A: External solutions have flexibility. Integration with data platform and AI platform (Elsa) expected. Diagram showing Elsa incorporation will be provided.
- Q: Technology consolidation across departments
 - A: Long-term goal is one platform for all tiers using same CRM tool. Assessment will determine best approach.
- Q: Tier 0 architecture and intake system
- A: Tier 0 focuses on clear self-service through website. Information about intake system design will be provided in RFP.
- Q: Customer experience goals and success metrics
 - A: Success means clear public reporting pathways, quick signal analysis, no delays, and ability to track all interactions across centers.

- Q: End user support at Tier 1
 - A: Focus is on external FDA intakes and complaints, not internal end-user support.
- Q: Signal detection scope
 - A: All signal detection in scope. Analytics component critical for identifying patterns across all intake channels.
- Q: Adverse event reporting integration
 - A: FAERS currently separate effort but could receive feeds from this system at Tier 3 level.
- Q: Year 1 expectations
 - A: Understanding entire environment, clear website with unified intake, CRM operational with analytics, system in place with rollout plan beginning.

V. Clarifications and Additional Information

- Technology and Integration:
 - FDA not married to single platform; seeking best-of-breed solutions
 - Existing systems include Genesis (telephony), Salesforce, Appian, ServiceNow
 - AI integration required with human-in-the-loop for decision-making
 - Unified analytics platform expected across all channels
 - Theme of consolidation across FDA operations
- Contracting Approach:
 - Statement of Objectives (SOO) to Performance Work Statement (PWS) approach
 - Vendors will write PWS based on SOO
 - Full and open competition
 - Small business participation encouraged
 - No preferred vendors or solutions
- Center-Specific Considerations:
 - CDRH: Access to combination product information across centers; accurate and expeditious information delivery
 - CDER: Case history visibility across centers; minimize delays
 - Foods & Cosmetics: Emergency escalation protocols; reporting capabilities; survey feedback integration
 - CVM: Small center needing translation of emails into structured data
 - Security: Compliance with NIST 800-53, FedRAMP standards
- Performance Expectations:
 - No dropped calls during transition
 - No delays in current service levels

- Service Level Agreements (SLAs) will be developed
- Key Performance Indicators (KPIs) open to vendor suggestions
- Both system-related and user outcome metrics
- Assessment Role:
 - Vendors will serve dual role as consultant and solution provider
 - Will assess existing solutions and create evaluation criteria
 - Government will make final decisions based on vendor recommendations
 - No baggage regarding existing systems or vendors

VI. Closing Remarks: Dr. Butler and Melissa Shea

- Emphasized level playing field for all vendors
- No sunk cost constraints
- Industry experience valued (vendors have done this at scale)
- Encouraged unconstrained thinking followed by practical implementation
- Focus on what's achievable in Year 1
- Questions will remain open via SAM.gov
- FDA moving quickly to finalize documents and release solicitation
- Additional industry days planned for future solicitations
- Feedback on forum format welcomed

VII. Next Steps

- Submit questions via SAM.gov
- Prepare for RFP release (60-90 days)
- Consider teaming arrangements
- Review requirements for PWS development
- FDA Actions:
 - Finalize solicitation documents
 - Post Q&A summary to SAM.gov
 - Release RFP in fiscal year 2026
 - Provide additional technical documentation (Elsa integration diagram, intake system architecture)
- Timeline:
 - Solicitation release: 60-90 days (target)
 - Melanie Keller assumes COO role: April 5, 2026
 - Dr. Butler transition to consultant role: Following April 5

VIII. Post-meeting

- Government personnel exited room to protect acquisition integrity

- Vendors provided networking opportunity until 1:00 PM
- Room remained available for vendor collaboration